IPAMORELIN

GH Secretagogue | Ghrelin Receptor Agonist | 5mg & 10mg x 10 Vials

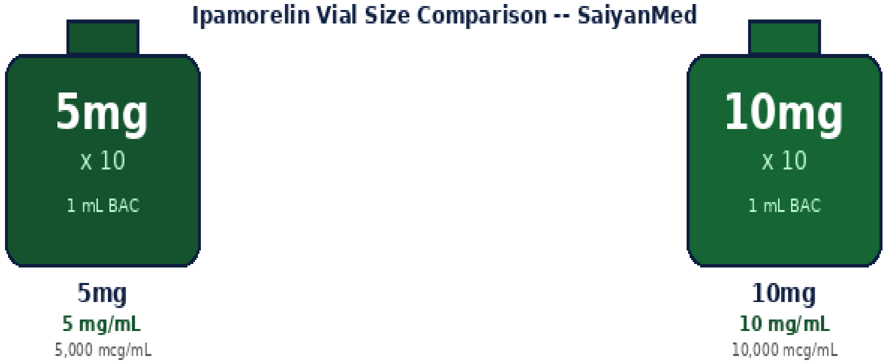
Clean GH Pulses -- No Cortisol -- No Prolactin -- Fasted SubQ Injection

Purity: 99.8% | Endotoxin-Free | Research Use Only

Ipamorelin is a selective pentapeptide GH secretagogue and ghrelin receptor agonist that stimulates the pituitary gland to release GH in clean, physiological pulses. Critically, it does NOT raise cortisol, prolactin, or ACTH — making it the cleanest GH secretagogue available. It requires fasted state for maximum effect and is highly synergistic with GHRH analogues (CJC-1295, Tesamorelin).

FASTED STATE IS MANDATORY: Insulin from food blunts GH release. Always inject at least 1-2 hours after your last meal. Wait at least 30 minutes before eating after injection.

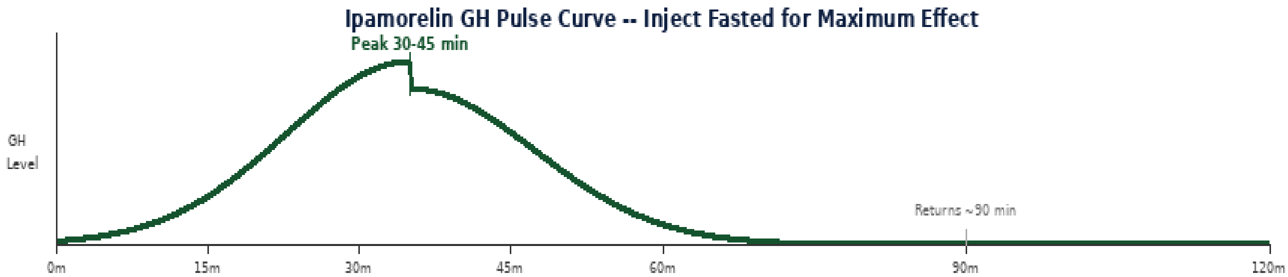
Ipamorelin Vial Size Comparison -- SaiyanMed



Parameter	Specification
Vial Sizes	5mg x 10 vials | 10mg x 10 vials | Purity: 99.8%, Endotoxin-Free
Reconstitution	1 mL BAC Water per vial (both sizes) | 5mg -> 5 mg/mL | 10mg -> 10 mg/mL
Standard Dose	100-300 mcg per injection | Start at 100 mcg and escalate gradually
Frequency	1-3x daily | Always fasted (1-2 hr after last meal)
Best Timing	Morning fasted + Bedtime (aligns with natural deep-sleep GH pulse)
GH Pulse	Peak at 30-45 min post-injection | Lasts ~60-90 min | Returns to baseline
Cycle	12 weeks on, 4 weeks off | Can be run year-round with breaks
Stack & Advantage	Stack with CJC-1295 or Tesamorelin (GHRH + GHS synergy) for 3-10x amplified GH pulse. No cortisol, no prolactin, no hunger -- cleanest GHS available.
Storage	2-8 deg C unreconstituted | Reconstituted: 2-8 deg C, use within 28 days.

MECHANISM OF ACTION & RESEARCH BENEFITS

Ipamorelin binds to the GHSR-1a (growth hormone secretagogue receptor / ghrelin receptor) on pituitary somatotroph cells, triggering selective GH release via the Gq/phospholipase C pathway. Unlike GHRP-2 and GHRP-6, it does not activate ACTH or cortisol pathways, and does not cause the hunger and cortisol spike seen with those peptides. GH is released in a clean pulse that mirrors the physiological pattern of natural GH secretion.



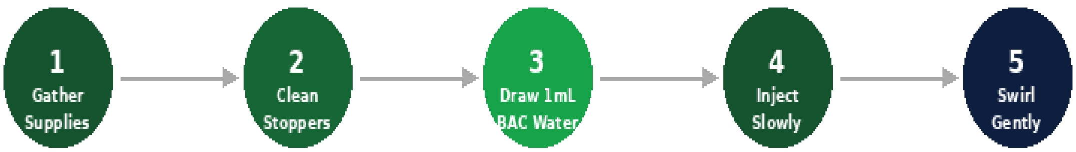
Pathway	Mechanism	Research Benefit
GHSR-1a Activation	Binds ghrelin receptor on pituitary somatotrophs; activates Gq protein and phospholipase C; raises intracellular IP3 and calcium; triggers exocytosis of GH from pituitary granules	Selective GH release without cortisol, prolactin, or ACTH co-stimulation
GH Pulse Amplification	Works synergistically with endogenous GHRH; when stacked with CJC-1295 or Tesamorelin, amplifies GH pulse 3-10x vs either agent alone; GHRH primes pituitary, GHS triggers release	Synergistic GH release; optimal with GHRH + GHS combination protocol
IGF-1 Elevation	GH released by Ipamorelin stimulates hepatic IGF-1 production; rises gradually over weeks of consistent use; supports anabolic signalling	Lean mass accretion, fat loss, recovery enhancement
Sleep Enhancement	Bedtime injection amplifies the natural GH surge during slow-wave (deep) sleep; GH and IGF-1 receptors in brain support neurogenesis; may improve sleep quality over time	Enhanced sleep-phase GH pulse; recovery during sleep
No Cortisol / Prolactin	Highly selective GHSR-1a agonism -- does NOT bind corticotroph cells; no ACTH/cortisol release; no prolactin elevation; no appetite stimulation -- key advantage over GHRP-2 and GHRP-6	Clean GH pulse with no hormonal side effects; suitable for long cycles

Key Research Benefits

Benefit	Evidence & Research Context
Cleanest GH Secretagogue Available	Ipamorelin is the most selective GHS -- no cortisol, no prolactin, no ACTH, no hunger. This allows higher doses and longer cycles vs GHRP-2/6.
GHRH Synergy	Combined with CJC-1295 or Tesamorelin, the GHRH primes somatotrophs while Ipamorelin triggers release -- amplifying the GH pulse 3-10x vs either alone.
Body Composition	Consistent GH elevation over 12-week cycles produces measurable fat reduction, lean mass preservation, and improved recovery alongside resistance training.
Sleep Quality	Bedtime injection amplifies the natural deep-sleep GH pulse, improving recovery, tissue repair, and over time, sleep architecture in research subjects.

RECONSTITUTION & DOSE CALCULATION

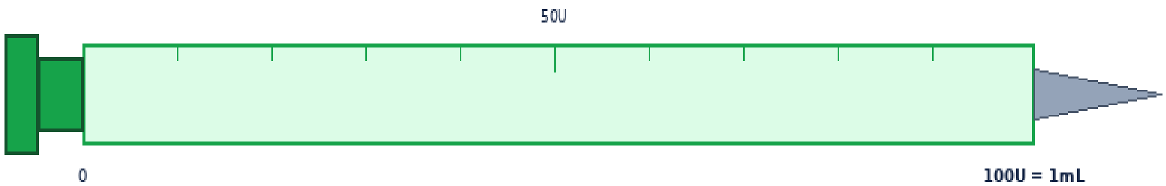
Both vial sizes (5mg and 10mg) use 1 mL BAC Water. The 5mg vial yields 5 mg/mL (5,000 mcg/mL). The 10mg vial yields 10 mg/mL (10,000 mcg/mL). Allow vial and BAC Water to reach room temperature before reconstitution. Never shake — roll gently until clear.



Both vials use 1 mL BAC Water. NEVER shake -- swirl gently.

Step	Action	Detail
1	Gather supplies	Ipamorelin vial, BAC Water vial, insulin syringes (29-31G), alcohol swabs, sharps container, date labels.
2	Clean vial stoppers	Wipe stoppers of BOTH vials with a fresh alcohol swab. Air-dry for 10-15 seconds.
3	Draw 1 mL BAC Water	Draw exactly 1 mL BAC Water. Remove all air bubbles. Use for both 5mg and 10mg vials.
4	Inject into Ipamorelin vial	Direct stream against the GLASS WALL -- never onto the powder. Inject slowly over 10-15 seconds.
5	Swirl gently	Roll between palms 20-30 seconds. NEVER shake. Solution should be clear and colourless within 60 seconds.
6	Label and refrigerate	Write reconstitution date. Store at 2-8 deg C. Use within 28 days. Never freeze.

DOSE CALCULATION GUIDE



Insulin Syringe -- 1mL / 100U | Use 29-31G needle | SubQ only

Formula: Volume (mL) = Desired Dose (mcg) / Concentration (mcg/mL) **|** **Units:** Volume (mL) x 100 = Units on syringe

Vial: 5mg x 10 -- Add 1 mL BAC Water → Concentration: 5.0 mg/mL (5,000 mcg/mL)

Dose (mcg)	100 mcg	150 mcg	200 mcg	250 mcg	300 mcg
Volume (mL)	0.0200	0.0300	0.0400	0.0500	0.0600
Syringe Units	2.0U	3.0U	4.0U	5.0U	6.0U
Doses / Vial	50	33	25	20	16

Vial: 10mg x 10 -- Add 1 mL BAC Water → Concentration: 10.0 mg/mL (10,000 mcg/mL)

Dose (mcg)	100 mcg	150 mcg	200 mcg	250 mcg	300 mcg
Volume (mL)	0.0100	0.0150	0.0200	0.0250	0.0300
Syringe Units	1.0U	1.5U	2.0U	2.5U	3.0U
Doses / Vial	100	66	50	40	33

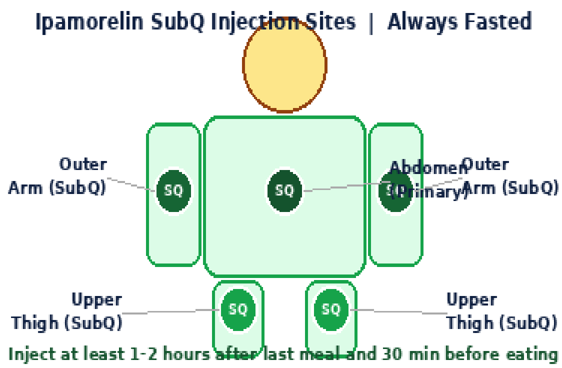
Timing rule: Inject within 30 minutes of waking (morning fasted) or 30 minutes before bed. Always at least 1-2 hours after last meal and 30 minutes before any caloric intake. The bedtime dose amplifies the natural deep-sleep GH pulse for enhanced overnight recovery.

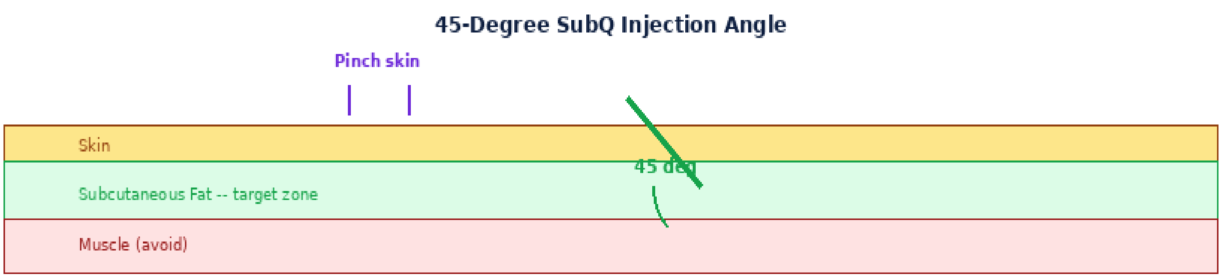
Step	Action	Detail
1	Confirm fasted state	At least 1-2 hours since last meal. Insulin blunts GH release -- fasting is mandatory.
2	Wash hands & gather supplies	Wash 20 seconds with soap. Dry completely. 29-31G insulin syringe (1mL or 0.3mL for doses under 150 mcg), Ipamorelin vial, alcohol swabs, sharps container.
3	Draw dose precisely	Draw exact volume per dose table. Expel all air bubbles. Volumes are small -- double-check measurement.
4	Clean site	Wipe chosen site with alcohol swab, circular motion. Allow 10-15 seconds to dry completely.
5	Pinch and inject	Pinch 2-3 cm skin. Insert at 45 degrees. Release pinch. Inject slowly over 5-10 seconds.
6	Withdraw, press & dispose	Remove at same angle. Press with dry swab 5-10 sec. Do NOT rub. Wait 30 min before eating. Record site, rotate systematically, needle into sharps container.

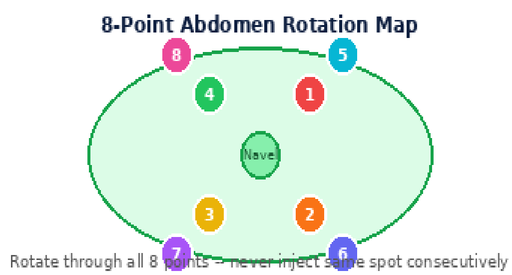
INJECTION SITES & ROTATION

Always inject SubQ (subcutaneously) into fat tissue at 45 degrees. Rotate systematically through 8 marked abdomen points to prevent lipoatrophy. Inject near the same general area each session for consistency. Allow at least 30 minutes post-injection before eating anything caloric.

Site	Route	Needle	Notes
Abdomen (2in from navel)	SubQ	29-31G 5/16 in	Primary site. 8-point rotation. Pinch skin, 45 degrees.
Outer upper arm	SubQ	29-31G 5/16 in	Good alternative. Pinch skin, 45 degrees.
Upper outer thigh	SubQ	29-31G 5/16 in	Alternate when abdomen sites need rest. No IM for Ipamorelin.



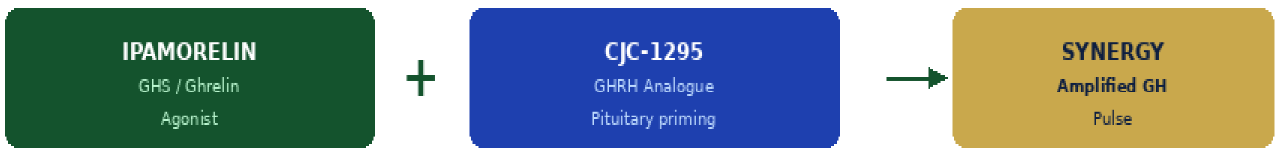




STACKING, PROTOCOLS, SIDE EFFECTS & GOLDEN RULES

GHRH + GHS Synergy Stack

GHRH + GHS Synergy -- Ipamorelin + CJC-1295 Stack



Inject both peptides simultaneously. Use separate syringes or combine in one.

Stack Partner	Class	Dose	Inject Together?	Effect
CJC-1295 (no DAC)	GHRH	100-300 mcg	Yes -- same time	Short-acting GHRH; matches Ipamorelin pulse; most common stack; amplifies GH 3-10x
Tesamorelin	GHRH	1,000-2,000 mcg	Yes -- same time	FDA-approved GHRH; strongest visceral fat reduction; premium stack for body recomposition
CJC-1295 + DAC	GHRH (lo ng-acting)	1-2 mg/week	Once weekly (DAC)	Long-acting GHRH; raises GH baseline; combine Ipamorelin for pulse on top of elevated baseline

Research Dosing Protocols

Protocol	Dose per Injection	Frequency	Duration	Notes
Beginner	100 mcg	1x daily (bedtime)	12 weeks on 4 weeks off	Start here. Bedtime only. Assess tolerance before adding morning dose.
Standard	200 mcg	2x daily (AM + bedtime)	12 weeks on 4 weeks off	Most common research protocol. Both doses fasted.
Advanced	300 mcg	3x daily (AM + noon + bed)	12 weeks on 4 weeks off	Maximises GH pulse frequency. All 3 doses must be fasted.
Stack (Standard)	200 mcg Ipa + 200 mcg CJC	2x daily	12 weeks on 4 weeks off	Inject both simultaneously. Best body composition results.

Side Effects

Side Effect	Likelihood	Notes
Injection site redness / itch	Common	Normal local reaction. Rotate sites. Resolves within 1-2 hours.
Mild headache (initial doses)	Occasional	Transient, first 1-2 weeks. Usually resolves. Ensure hydration.
Water retention (mild)	Occasional	GH-mediated. More pronounced at 300 mcg. Reduce dose if significant.
Tingling (hands / feet)	Occasional	GH effect on nerve tissue. Common at higher doses. Usually mild and transient.
Fatigue / drowsiness	Common (bedtime)	Expected at bedtime dose -- GH promotes deep sleep. Inject 30 min before bed.

No cortisol, prolactin, or hunger	N/A (advantage)	Unlike GHRP-2/6, Ipamorelin does NOT cause cortisol spike, prolactin elevation, or appetite stimulation.

Contraindications: Active cancer or malignancy history (GH/IGF-1 stimulation), pregnancy or lactation. Not for use under 18. Monitor for signs of GH excess (acromegaly) with prolonged high-dose use.

Storage Guidelines

State	Temperature	Duration	Notes
Unreconstituted powder	2-8 deg C	Per expiry	Keep upright. Away from light. Do not freeze.
Reconstituted solution	2-8 deg C	28 days	Label with reconstitution date. Inspect before each use.
BAC Water (opened)	2-8 deg C	28 days	Label with opening date.
Transport	Below 25 deg C	Max 72 hr	Insulated bag with ice packs. No direct ice contact.

Golden Rules -- Ipamorelin

1. ALWAYS inject fasted -- at least 1-2 hours after last meal. Insulin from food kills the GH pulse.
2. Both vial sizes (5mg and 10mg) use exactly 1 mL BAC Water for reconstitution.
3. Start at 100 mcg once daily (bedtime) and escalate gradually over 2-4 weeks.
4. Bedtime is the optimal timing -- inject 30 min before sleep to amplify deep-sleep GH pulse.
5. Never shake the vial -- roll gently between palms for 20-30 seconds until clear.
6. Stack with CJC-1295 or Tesamorelin for a 3-10x amplified GH pulse via GHRH + GHS synergy.
7. Inject both stack peptides simultaneously at the same site or separate sites.
8. Wait at least 30 minutes post-injection before consuming any calories.
9. Rotate injection sites through all 8 abdomen points -- never reuse same spot consecutively.
10. Cycle: 12 weeks on, 4 weeks off -- mandatory break to preserve pituitary sensitivity.